

Discharge Planning and Care Transitions

Post-Acute Care Coordination and Readmission
Prevention

Regulation: 42 CFR §482.43 | §482.13(b)

POLICY OVERVIEW

Effective discharge planning reduces hospital readmissions, ensures continuity of care, and supports patient recovery in the community. CMS requires that discharge planning begin early in the hospital stay and actively involve patients and families in determining appropriate post-acute care.

KEY REQUIREMENTS

1 Early Discharge Planning Initiation

Discharge planning must begin within 24 hours of admission for all patients. A Registered Nurse or social worker must conduct a discharge needs assessment including: living situation, support systems, functional status, home safety, and financial resources.

2 Multidisciplinary Collaboration

The discharge planning process must involve all relevant disciplines: nursing, social work, case management, physical/occupational therapy, pharmacy, and the attending physician. Multidisciplinary rounds should include discharge planning as a standing agenda item.

3 Patient and Family Involvement

The patient and/or designated representative must be involved in all discharge planning decisions. The planned discharge destination must be discussed with the patient, and the patient's preferences must be considered. Patients have the right to request a Quality Improvement Organization (QIO) review before discharge.

4 Transition of Care Documentation

The discharge summary must be completed and transmitted to the receiving provider within 24 hours of discharge (or within 48 hours for complex patients). It must include: diagnosis, hospital course, medications, follow-up appointments, and pending test results.

5 Readmission Risk Reduction

High-risk patients (LACE score or equivalent) must receive enhanced transition services including: post-discharge telephone follow-up within 48-72 hours, medication reconciliation at discharge, confirmed follow-up appointment, and patient education on red-flag symptoms.

COMPLIANCE NOTES

CMS Hospital Readmissions Reduction Program (HRRP) penalizes hospitals with excess readmissions for AMI, CHF, pneumonia, COPD, hip/knee arthroplasty, and CABG. A transitions of care program with community partnerships is essential for readmission reduction.

COMMON SURVEY CITATIONS

Required under §482.43 – Discharge planning requirements, and subject to CMS HRRP penalties for excess readmissions.